

Congenital Heart Disease — Revision Table			
Lesion	Shunt/Type	Key fact	Surgery & timing
PDA	L→R (17%)	Aorta↔proximal LPA	Close after 1st mo; any time if HF; CI if severe PVD
ASD	L→R (16.5%)	Most common malformation	Repair if Qp/Qs>1.5–2; age 1–5
VSD	L→R (13%)	Hole between ventricles	Large+CHF: <3mo; small: observe (IE risk)
AVSD	L→R (3.5%)	Endocardial cushion defect	Partial 1–2yr; complete 3–6mo
Coarctation	Obstructive (9.5%)	Narrow upper thoracic aorta	Lumen↓>50%, HTN, CHF, +VSD
Pulmonary stenosis	Obstructive	RVOT obstruction	Sx + gradient >50mmHg; balloon valvotomy
TOF	R→L (4.5%)	RV infundibulum underdev., cyanotic	Dx = indication; total correction 3–24mo
TGA	Admixture (5%)	Aorta off RV, PA off LV	Arterial switch <1mo; later → atrial switch
Tricuspid atresia	R→L (3%)	Univentricular AV connection	Shunt → BCPS 6–12mo → Fontan 12–24mo
HLHS	(0.9%)	Hypoplastic left structures	Norwood 1–30d → BCPS → Fontan
Ebstein	Valvular	TV leaflets displaced into RV	Repair + ASD closure if symptomatic; WPW ablation
Palliative op	Purpose		Used in
Blalock-Taussig shunt	↑ pulmonary flow		Severe cyanosis, low lung flow
PA banding	↓ pulmonary flow		Large/multiple VSD, CHF
Atrial septectomy	↑ mixing		TGA, tricuspid/pulmonary atresia
Cause group	Examples		
Genetic (10%)	Chromosomal (Trisomy 21), single gene		
Teratogen drugs	Alcohol, thalidomide, retinoic acid, anticonvulsants, lithium		
Teratogen infection	Rubella, Coxsackie B		
Maternal	Diabetes (VSD/TGA), SLE (heart block), PKU, old age		
Transplant	Detail		
Indication	Poor prognosis <1yr, not treatable conventionally		
Contraindication	Severe coexisting disease, active infection, PVR >8 Wood units		
<i>Bold = high-yield.</i>			

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